



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : منال عطيه نوار ميخائيل
المحافظة : الجيزه
الحى / المدينة : الجيزه
رقم البطاقة : 27806152501885 رقم التليفون : 01280019575
العنوان : مكى
العمر (تاريخ الميلاد) : 15-06-1978 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 05-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي تاموكسيفين علاج هرموني [14160] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- / د. / ----- / د. / ----- / د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: منال عطيه نوار ميخائيل National ID: 27806152501885	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Tamoxifen hormonal therapy.
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

منال
عطيه نوار ميخائيل
٨ ش الاتحاد - ساقية مكي
الجيزة - الجيزة

٢٧٨٠٦١٥٢٥٠١٨٨٥

KU0808871

٢٠٢٤/١٠ ٢٧٨٠٦١٥٢٥٠١٨٨٥

حاصله على دبلوم تجاره

أنثى
مسجلة
متزوجة
مسعد فؤاد غطاس بحيث
البطاقة سارية حتى ٢٠٣١/٠٩/٣٠

١



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

السلطة المصرية
صحة
مصر

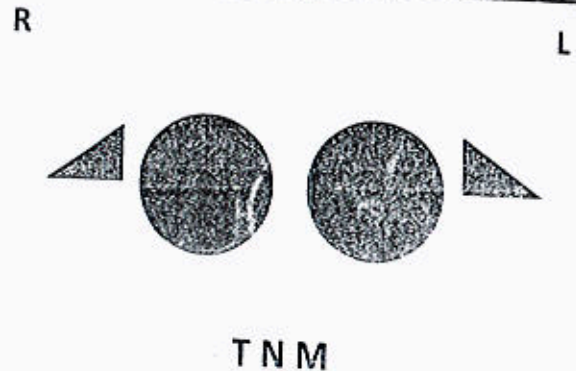
MDT FORM

12/6/2025 G

Name	منال عطية نوار	MRN	4170/9.52
Age	47	ID	13521779
Menopausal status		Phone	0120-19570
NRS SCORE		Marital status	24.7 1000.180
BMI		Family history	-ve
CO morbidity	NAD	REASON OF MDT	Post-operative

Clinical Data

BX:	IDC		
ER:	PR:	HER 2:	Ki67:
3/8	4/8	+3	45%
Sonomammography:			
Patient underwent Rt. WLE + SLNB			
Mets Workup:			
- Surgical margins: free			
- SLNB: 0/8			



Recommendations:

- ① PORT
- ② Continue Adjuvant phesgo (12 cycles)
- ③ Hormonal th (TAM)

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. خالد عبد الحزير					

SMC 004-MDT-001

Revision no:0

Issue no:1

Issue date: 3/2025

Name: منال عطيه نوار ميخائيل			
MR. : 6967			
Detailed Medical Report			
		السيد	
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
رقم طبي:		بقسم:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الخروج :		تاريخ وساعة الدخول :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
46YRS, WEAK HORMONAL ER4\8 PR 3\8 HER2+3 TCHP >> RT WLE 5-2025 >. PORT + PHESGO + TAM ECHO 17-11-2025>> 65% C14 PHESGO + TAM C15+ C16(ECHO) >> PHESGO+ TAM >> PHARMACY			
Clinical pharmacy note : for C15 phesgo for echo next visit			
Pharmacy Visit FOr Cycle 16 Phesgo + Tamoxifen ECHO 15/3 ... 60%			
The patient has completed 16 cycles of PHESGO + TAM.			
تحريرا في ,			
التاريخ :		20-09-2026	
الوقت :		اسماء محمد مصطفى نصر	
		الطبيب المعالج :	
يعتمد ,			

PATIENT NAME: MANAL ATEA NAWWAR Makaal

GENDER: Female

DATE : 30-Nov-2024 02:01 PM

PATIENT ID: 10124254110

AGE: 46 y

ORDER: 3139626,3145163

**CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND EXAMINATION OF BOTH
BREASTS**

INDICATION:

- Right Breast pathologically proved carcinoma outside baheya.

TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO views
- Complementary us.
- IV non-ionic contrast medium infusion 100 ml of omnipaque

REPORT:

- Scattered fibroglandular parenchyma breasts (ACR B).

Right breast :

- Right UIQ spiculated hyperdense mass, corresponding by U/S to right UIQ 1-2 o'clock irregular spiculated heterogenous hypoechoic mass measuring about 27x17mm with surrounding echogenic halo and posterior shadowing. It is seen about 3.7mm from the overlying skin and more than 60mm away from the nipple, No evidence of gross involvement of the pectoral muscle.
- No suspicious micro calcific clusters.

Contrast Enhanced Spectral Mammography showed :

- heterogenous pathological mass enhancement of the fore mentioned 1-2 o'clock mass, measuring 44x38mm. About 67mm from the nipple (measured in RT CC).
- Normal skin thickness and contour.
- Normal nipple areola complex.
- All examined axillary lymph nodes show preserved hila and average cortical thickness, non-specific.
- Free supra-clavicular region.

PATIENT: MANAL ATEA NAWWAR Makaal, 10124254110, Female, 46 y

Report version 2, 30-Nov-2024 02:53 PM

Page 1 of 2

Left breast:

- No spiculated mass lesions or suspicious microcalcific clusters.

Contrast Enhanced Spectral Mammography showed:

- Minimal back ground parenchymal enhancement.
- No suspicious solid lesions.
- Normal skin thickness and contour.
- Normal nipple areola complex.
- Non-specific axillary lymph nodes.

OPINION:

Right Breast pathologically proven carcinoma.

- ***Right Breast UIQ pathologically enhancing malignant-looking solid mass as described (BIRADS 6).***
- ***Left Breast no pathological mass or non mass enhancement (BIRADS 2).***

Reported By,

Dr. Nehal Hamdey, MSc, FFBR

30-Nov-2024 02:53 PM

PATIENT: MANAL ATEA NAWWAR Makaal, 10124254110, Female, 46 y

Report version 2, 30-Nov-2024 02:53 PM

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Patient Name :	الأستاذة / منال عطية نوار ميخائيل	Request No. :	13008811
Patient No. :	27806152501885	Request Date :	15/10/2024
Path No. :	3928/24	Verification Date :	28/10/2024
Sex :	Female	Financial Category :	اورام دار السلام - نفقة الدولة
Age :	46 Y	Department :	Out Patient
Referring Doc :		Registered by :	.marwa mohamed youss

Pathology Report

IPT:

- ER: Positive (score 3/8).
- PR: Positive (score 4/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 45% of tumor cells.

* Comment:

The cancer in this sample has a low level (1-10%) of ER expression by IHC. There are limited data on the overall benefit of endocrine therapies for patients with these results, but they currently suggest possible benefit, so patients are considered eligible for endocrine treatment. There are data that suggest invasive cancers with these results are heterogenous in both behaviour and biology and often have gene expression profiles more similar to ER-negative cancers.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Verified by :

dr.esraa magdy

Printing Date : 04/11/2024 10:52:24

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذة / منال عطيه نوار ميخائيل	Request No.	: 13008811
Patient No.	: 27806152501885	Request Date	: 15/10/2024
Path No.	: 3928/24	Verification Date	: 28/10/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 46 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Signature
Dr. Rana Fathy.
Dr. Esraa Magdy.

Verified by :
dr.esraa magdy



Women's Health Outreach Program

Name: Manal Atia Nawar

Date: 14-10-2024

Bilateral Full Field Digital Mammography

Clinical summary: 46 years old, seen for a palpable right breast lump felt over the past few months

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Scattered fibro glandular densities (ACR density B).
- An irregular mass of high density is noted at the UIQ posterior third with associated architectural distortion seen extending anterior to the mass in the middle third.
- No suspicious calcifications.
- Ultrasound demonstrated a corresponding irregular inhomogeneous hypoechoic solid mass seen palpable at 2 o'clock overlying the chest wall measuring 1.7 x 1.4 x 1.4 cm with ductal extension of 1.2 cm seen extending anterior to the mass likely corresponding to mammography distortion. Doppler ultrasound showed a prominent feeding vessel. AI score 10 (highly suspicious).
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Left breast

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated no solid or cystic lesion
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

Right breast (BIRADS 4c): highly suspicious palpable non calcified solid mass at the UIQ, as described

Left breast (BIRADS 1): Normal study

Recommendations:

Biopsy was performed in today sitting for the right breast mass for correlation and surgical consultation.

Dr: Iman Ewa's
Breast Imaging Specialist



Patient Name: Marial Atteya Nawwar Mikhail	Date : 05/11/2024
Exam : C.T CHEST	Age : 46 Year(s)
Medical ID: 3865/2024	Pat. ID: 93933

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Right breast UIQ irregular lesion measuring 1.7 cm.
- Bilateral prominent axillary lymph nodes (right level I& II and left level I) the largest at right side measuring 1.5 X 2 cm for sonographic correlation.
- Prominent mediastinal lymph nodes the largest measured **0.9 cm.**
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No definite enhancing focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus for regional US assessment
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:**



Patient Name: Manal Atteya Nawwar Mikhail	Date : 05/11/2024
Exam : C.T CHEST	Age : 46 Year(s)
Medical ID: 3865/2024	Pat. ID: 93933

- Sclerotic lesion is seen at left acetubulum likely bone island for follow up
- no definite worrisome lytic focal lesions.

Opinion:

- Prominent bilateral axillary lymph nodes for sonomammography correllation
- No evidence of pulmonary or hepatic metastatic deposits on non-contrast basis.

Dr. Mai Salem

Dr. Mohamed Mohsen, M.D.

Patient Name : الأستاذة / منال عطيه نوار ميخائيل	Request No. : 13003169
Patient No. : 27806152501885	Request Date : 08/05/2025
Path No. : 1662/25	Verification Date : 21/05/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 46 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, post NACT.
Nature Specimen: Right WLE and SLNs.

Gross Examination:

- * Right WLE specimen measured 5x4 cm, with covering skin ellipse measured 4x1 cm.
- * Tumor extent: A wired rubbery grayish white tumor bed site.
- Size: 1.5x1 cm.
- * Distance from margins: 0.4 cm from least deep margin.
- * Right SLN: A fibro fatty tissue piece measured 4x3 cm entangling 8 nodules up to 1x1 cm.
- * Frozen sections result:
- Free margins.
- Free sentinel lymph nodes (0/6), 2 lymph nodes left for paraffin.
- * Designation:
- 1: Medial margin.
- 2: Lateral margin.
- 3: Superior margin.
- 4: Inferior margin.
- 5: Deep margin.
- 6: Frozen sentinel lymph nodes.
- 7: Paraffin sentinel lymph nodes.
- 8-9: Tumor bed site.

Dissected by: Dr. Heba Hassan.

Microscopic Examination:

- * Right breast, WLE and SLNs biopsy:
- * Histological Type: No residual tumor.
- * Ductal carcinoma in situ: Not detected.
- * Lymph node involvement: Free sentinel lymph nodes (0/8).

Verified by :

Dr.Toka Medhat



Patient Name : الأستاذة / منال عطيه نوار ميخائيل	Request No. : 13003169
Patient No. : 27806152501885	Request Date : 08/05/2025
Path No. : 1662/25	Verification Date : 21/05/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 46 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

- * Angioinvasion: Not detected.
- * Perineural invasion; Not detected.
- * Resection Margins: Free surgical resection margins.
- * Histologic response to therapy: Complete pathological response to therapy.
- * Microcalcifications: Not detected.

Diagnosis:

Right breast, WLE and SLNs Biopsy:

No residual tumor.

- Free surgical resection margins.
- Free sentinel lymph nodes (0/8).
- Complete pathological response to therapy.

Stage: ypT0, ypN0.

Signature
Dr. Walaa Ghonaim
Dr. Yasmine Essam

Verified by :

Dr.Toka Medhat

Printing Date : 22/05/2025 10:49:17

Page 2 of 2

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